



PRODUCT NAME
DOSANAC INJECTION

NAME AND STRENGTH OF ACTIVE SUBSTANCE
Each 3 mL ampoule contains 75 mg of diclofenac sodium.

PRODUCT DESCRIPTION
Clear, colourless sterile solution in clear glass ampoule.

PHARMACODYNAMICS

Mechanism of action (MOA)

DOSANAC contains diclofenac sodium, a non-steroidal compound with pronounced antirheumatic, anti-inflammatory, analgesic, and antipyretic properties. Inhibition of prostaglandin biosynthesis, which has been demonstrated in experiments, is considered to be fundamental to its mechanism of action. Prostaglandins play an important role in causing inflammation, pain and fever. Diclofenac sodium in vitro does not suppress proteoglycan biosynthesis in cartilage at concentrations equivalent to the concentrations reached in humans.

Pharmacodynamics (PD)

In rheumatic diseases, the anti-inflammatory and analgesic properties of DOSANAC elicit a clinical response characterized by marked relief from signs and symptoms such as pain at rest, pain on movement, morning stiffness, and swelling of the joints, as well as by an improvement in function. In post-traumatic and post-operative inflammatory conditions, DOSANAC rapidly relieves both spontaneous pain and pain on movement and reduces inflammatory swelling and wound edema. DOSANAC has also been found to exert a pronounced analgesic effect in moderate and severe pain of non-rheumatic origin, an effect which sets in within 15 to 30 minutes. When used concomitantly with opioids for the management of post-operative pain, DOSANAC significantly reduces the need for opioids. DOSANAC ampoules are particularly suitable for initial treatment of inflammatory and degenerative rheumatic diseases, and of painful conditions due to inflammation of non-rheumatic origin.

PHARMACOKINETICS

Absorption

Intramuscular injection: When 75 mg diclofenac is administered intramuscularly, absorption sets in immediately and achieved mean peak plasma concentrations after about 20 minutes ($C_{max} = 2.5 \mu\text{g/mL}$). The amount absorbed is in linear proportion to the size of the dose.

Intravenous infusion: When 75 mg diclofenac is administered via intravenous infusion for 2 hours, mean peak plasma concentrations are approximately $1.9 \mu\text{g/mL}$. Shorter infusions result in higher peak plasma concentrations, while longer infusions give plateau concentrations proportional to the infusion rate after 3-4 hours.

Distribution

The active substance is 99.7% protein bound, mainly to albumin (99.4%). Diclofenac enters the synovial fluid, where maximum concentrations are measured 1-2 hours after the peak plasma values have been attained. The apparent half-life for elimination from the synovial fluid is 3-6 hours. Two hours after reaching the peak plasma values, concentrations of the active substance are already higher in the synovial fluid than they are in the plasma and remain higher for up to 12 hours. Diclofenac was detected in a low concentration (100 ng/mL) in breast milk in one nursing mother. The estimated amount ingested by an infant consuming breast milk is equivalent to a 0.03 mg/kg/day dose.

Metabolism

Biotransformation of diclofenac takes place partly by glucuronidation of the intact molecule, but mainly by single and multiple hydroxylation and methoxylation, resulting in several phenolic metabolites, most of which are converted to glucuronide conjugates. Two phenolic metabolites are biologically active, but to a much lesser extent than diclofenac.

Excretion

The terminal half-life of diclofenac in plasma is 1-2 hours. Four of the metabolites, including the two active ones, also have short plasma half-lives of 1-3 hours. About 60% of the administered dose is excreted in urine as the glucuronide conjugate form of the intact molecule and as metabolites, most of which are also converted to glucuronide conjugates. Less than 1% is excreted as unchanged substance. The rest of the dose is eliminated as metabolites through the bile in faeces.

INDICATIONS

Intramuscular injection

- Exacerbations of inflammatory and degenerative forms of rheumatism (rheumatoid arthritis, ankylosing spondylitis, osteoarthritis, spondylarthritis, painful syndromes of the vertebral column, non-articular rheumatism)

- Acute attacks of gout
- Renal colic and biliary colic
- Post-traumatic and post-operative pain, inflammation and swelling

Intravenous infusion

- Treatment or prevention of post-operative pain in a hospital setting

DOSAGE AND ADMINISTRATION

Dosage

As a general recommendation, the dose should be individually adjusted. Adverse effects may be minimized by using the lowest effective dose for the shortest duration necessary to control symptoms (see section WARNINGS AND PRECAUTIONS).

General target population: adults

DOSANAC solution for injection should not be given for more than 2 days; if necessary, treatment can be continued with DOSANAC tablets or suppositories (see METHOD OF ADMINISTRATION).

Special populations

Paediatric patients (below 18 years of age)

Because of their dosage strength, the ampoules of DOSANAC solution for injection are not suitable for children and adolescents.

Geriatric patients (aged 65 years or above)

No adjustment of the starting dose is generally required for elderly patients. However, caution is indicated on basic medical grounds, especially for frail elderly patients or those with a low body weight (see section WARNINGS AND PRECAUTIONS).

Established cardiovascular disease or significant cardiovascular risk factors

Treatment with DOSANAC is generally not recommended in patients with established cardiovascular disease (congestive heart failure, established ischemic heart disease, peripheral arterial disease) or uncontrolled hypertension. If needed, patients with established cardiovascular disease, uncontrolled hypertension, or significant risk factors for cardiovascular disease (e.g. hypertension, hyperlipidaemia, diabetes mellitus and smoking) should be treated with DOSANAC only after careful consideration and only at doses $\leq 150 \text{ mg}$ daily when initial treatment with DOSANAC solution for injection continues with DOSANAC tablets or suppositories for more than 4 weeks (see section WARNINGS AND PRECAUTIONS).

Renal impairment

DOSANAC is contraindicated in patients with renal failure ($\text{GFR} < 15 \text{ mL/min/1.73m}^2$) (see section CONTRAINDICATIONS).

No specific studies have been carried out in patients with renal impairment, therefore, no specific dose adjustment recommendations can be made. Caution is advised when administering DOSANAC to patients with renal impairment (see section WARNINGS AND PRECAUTIONS).

Hepatic impairment

DOSANAC is contraindicated in patients with hepatic failure (see section CONTRAINDICATIONS).

No specific studies have been carried out in patients with hepatic impairment, therefore, no specific dose adjustment recommendations can be made. Caution is advised when administering DOSANAC to patients with mild to moderate hepatic impairment (see section WARNINGS AND PRECAUTIONS).

Method of administration

Intramuscular injection

The following directions for intramuscular injection must be followed in order to avoid damage to a nerve or other tissue at the injection site (which may result in muscle weakness, muscle paralysis, hyposthesia and Embolia cutis medicamentosa (Nicolau syndrome)).

The dose is generally one 75 mg ampoule daily, given by deep intragluteal injection into the upper outer quadrant using aseptic technique. In severe cases (e.g. colic), the daily dose can exceptionally be increased to two injections of 75 mg, separated by an interval of a few hours (one into each buttock). Alternatively, one ampoule of 75 mg can be combined with other pharmaceutical forms of DOSANAC (e.g. tablets, suppositories) up to a total maximum daily dose of 150 mg.

Intravenous infusion

DOSANAC solution for injection must not be given as an intravenous bolus injection.

Immediately before starting an intravenous infusion, DOSANAC solution for injection must be diluted with saline 0.9% or glucose 5% infusion solution buffered with sodium bicarbonate according to the instructions given in section INSTRUCTIONS FOR USE AND HANDLING.

Two alternative dosage regimens of DOSANAC solution for injection are recommended.

For the treatment of moderate to severe post-operative pain, 75 mg should be infused continuously over a period of 30 minutes to 2 hours. If necessary, treatment may be repeated after a few hours,

DOSANAC[®] Injection

IM/IV Infusion

but the dose should not exceed 150 mg within any period of 24 hours.

For the prevention of post-operative pain, a loading dose of 25 mg to 50 mg should be infused after surgery over 15 minutes to 1 hour, followed by a continuous infusion of about 5 mg per hour up to a maximum daily dose of 150 mg.

CONTRAINDICATIONS

Diclofenac is contraindicated in circumstances as below:

- Known hypersensitivity to the active substance, sodium metabisulphite or any of the other excipients
- Active gastric or intestinal ulcer, bleeding or perforation (See *Warnings and Precautions*)
- Last trimester of pregnancy (See *Pregnancy and Lactation*)
- Hepatic failure
- Renal failure ($\text{GFR} < 15 \text{ mL/min/1.73m}^2$)
- Severe cardiac failure (See *Warnings and Precautions*)
- Like other NSAIDs, diclofenac is contraindicated in patients who are susceptible to NSAIDs-induced cross-reactivity reactions, where the use of acetylsalicylic acid or other NSAIDs can precipitate asthma, angioedema, urticaria, or acute rhinitis (See *Warnings and Precautions*)

WARNINGS AND PRECAUTIONS

WARNINGS

RISK OF GI ULCERATION, BLEEDING, AND PERFORATION WITH NSAID

Serious GI toxicity such as bleeding, ulceration and perforation can occur at any time, with or without warning symptoms, in patients treated with NSAID therapy. Although minor upper GI problems (e.g., dyspepsia) are common, usually developing early in therapy, prescribers should remain alert for ulceration and bleeding in patients treated with NSAIDs even in the absence of previous GI tract symptoms.

Studies to date have not identified any subset of patients not at risk of developing peptic ulceration and bleeding. Patients with prior history of serious adverse events and other risk factors associated with peptic ulcer disease (e.g., alcoholism, smoking and corticosteroid therapy) are at increased risk. Elderly or debilitated patients seem to tolerate ulceration or bleeding less than other individuals and account for most spontaneously reports for fatal GI events.

Gastrointestinal effects

GI bleeding ulceration or perforation, which can be fatal, have been reported with all NSAIDs (including diclofenac), and may occur at any time during treatment, with or without warning symptoms or a previous history of serious GI events. They generally have more serious consequences in the elderly. Diclofenac treatment should be discontinued if GI bleeding or ulceration is observed.

As with all NSAIDs (including diclofenac), close medical surveillance is imperative and particular caution should be exercised when diclofenac is prescribed to patients with symptoms indicative of GI disorders or with a history suggestive of gastric or intestinal ulceration, bleeding or perforation. The risk of GI bleeding is higher with increasing NSAID doses and in patients with a history of ulcer, particularly if complicated with haemorrhage or perforation and in the elderly.

To reduce the risk of GI toxicity in patients with a history of ulcer, particularly if complicated with haemorrhage or perforation, and in the elderly, the treatment should be initiated and maintained at the lowest effective dose.

Combination therapy with protective agents (e.g., proton pump inhibitors or misoprostol) should be considered for these patients, and also for patients requiring concomitant use of low-dose acetylsalicylic acid, or other drugs likely to increase GI risk.

Patients with a history of GI toxicity, particularly the elderly, should report any unusual abdominal symptoms (especially GI bleeding). Caution is recommended in patients receiving concomitant medications which could increase the risk of ulceration or bleeding, such as systemic corticosteroids, anticoagulants, anti-platelet agents or selective serotonin-reuptake inhibitors (See *Interactions*). Close medical surveillance and caution should also be exercised in patients with ulcerative colitis or Crohn's disease as their condition may be exacerbated.

NSAIDs, including diclofenac, may be associated with increased risk of gastro-intestinal anastomotic leak. Close medical surveillance and caution are recommended when using diclofenac after GI surgery.

Cardiovascular effects

Treatment with NSAIDs including diclofenac, particularly at high dose and in long term, may be associated with an increased risk of serious cardiovascular thrombotic events (including myocardial infarction and stroke).

Treatment with diclofenac is generally not recommended in patients with established cardiovascular disease (congestive heart failure, established ischemic heart disease, peripheral arterial disease) or uncontrolled hypertension. If needed, patients with established cardiovascular disease, uncontrolled hypertension, or significant risk factors for cardiovascular disease (e.g. hypertension, hyperlipidaemia, diabetes mellitus and smoking) should be treated with diclofenac only after careful consideration and only at doses $\leq 100 \text{ mg}$ daily when continues for more than 4 weeks.

As the cardiovascular risks of diclofenac may increase with dose and duration of exposure, the lowest effective daily dose should be used for the shortest duration possible. The patient's need for symptomatic relief and response to therapy should be re-evaluated periodically, especially when treatment continues for more than 4 weeks.

Patients should remain alert for the signs and symptoms of serious arteriothrombotic events (e.g. chest pain, shortness of breath, weakness, slurring of speech), which can occur without warnings. Patients should be instructed to see a physician immediately in case of such an event.

Haematologic effects

Diclofenac may temporarily inhibit platelet aggregation, patients with defects of haemostasis should be carefully monitored. As with other NSAIDs (including diclofenac), monitoring of blood count is recommended during prolonged treatment.

Respiratory effects (pre-existing asthma)

In patients with asthma, seasonal allergic rhinitis, swelling of the nasal mucosa (nasal polyps), chronic obstructive pulmonary diseases or chronic infections of the respiratory tract (especially if linked to allergic rhinitis-like symptoms), reactions on NSAIDs-like asthma exacerbations (intolerance to analgesics-asthma), Quincke's oedema or urticaria are more frequent in such patients, hence special caution is recommended in such patients (readiness for emergency). This is applicable as well for patients who are allergic to other substances, e.g., with skin reactions, pruritus or urticaria.

Special caution is recommended when diclofenac is used parenterally in patients with bronchial asthma as their symptoms may be exacerbated.

Hepatobiliary effects

Close medical surveillance is required when prescribing diclofenac to patients with impaired hepatic function as their conditions may be exacerbated. Diclofenac may cause elevation of one or more liver enzymes.

As with other NSAIDs (including diclofenac), monitoring of hepatic function is indicated as a precautionary measure during prolonged treatment. Diclofenac treatment should be discontinued, if abnormal liver function tests persist or worsen, clinical signs or symptoms consistent with liver disease develop, or if other manifestations occur (e.g. eosinophilia, rash). Hepatitis may occur with use of diclofenac without prodromal symptoms.

Caution is recommended when diclofenac is used in patients with hepatic porphyria as it may trigger an attack.

Skin reactions

Serious skin reactions, some of them fatal (e.g., exfoliative dermatitis, Stevens-Johnson syndrome, toxic epidermal necrolysis (Lyell's syndrome)) have been reported very rarely in association with the use of NSAIDs (including diclofenac). Patients appear to be at highest risk of these reactions in the early course of therapy, where the onset of the reaction occurs within the first month of treatment in majority cases. Diclofenac treatment should be discontinued at the first appearance of skin rash, mucosal lesions, or any other sign of hypersensitivity.

As with other NSAIDs, allergic reactions (anaphylactic/anaphylactoid reactions) can also occur in rare cases with or without earlier exposure to the drug.

Renal effects

As fluid retention and oedema have been reported in association with NSAID therapy (including diclofenac), caution is recommended when diclofenac is used in patients with impaired cardiac or renal function, history of hypertension, the elderly, patients receiving concomitant treatment with diuretics or medicinal products that can significantly impact renal function, and in those with substantial extracellular volume depletion of any cause (e.g., before or after major surgery). Monitoring of renal function is indicated as a precautionary measure in such cases, discontinuation of therapy is usually followed by recovery to the pre-treatment state.

Injection site reactions

Injection site reactions have been reported after the administration of Diclofenac intramuscularly, including injection site necrosis and embolia cutis medicamentosa, also known as Nicolau Syndrome (particularly after subcutaneous administration). Appropriate needle selection and injection technique should be followed during IM administration of Diclofenac (see *Instruction for Use and Handling*).

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Geriatric patients

Caution is indicated in the elderly on basic medical grounds, especially in frail elderly patients or those with a low body weight.

Interaction with NSAIDs

The concomitant use of diclofenac with systemic NSAIDs, including cyclooxygenase-2 selective inhibitors should be avoided due to undesirable effects.

Special excipient

This preparation contains sodium metabisulphite that may cause serious allergic type reactions in certain susceptible patients. Do not use if known to be hypersensitive to bisulphites.

As this preparation contains benzyl alcohol, its use should be avoided in children under two years of age.

Not to be used in neonates.

Masking signs of infections

Like other NSAIDs, diclofenac may mask the signs and symptoms of infection due to its pharmacodynamic properties.

INTERACTIONS

The following interactions include those observed with diclofenac solution for injection and/or other pharmaceutical forms of diclofenac.

CYP2C9 inhibitors

Caution is recommended when co-prescribing diclofenac with CYP2C9 inhibitors (such as voriconazole), which could result in a significant increase in peak plasma concentrations and exposure to diclofenac.

Lithium

If used concomitantly, diclofenac may increase plasma concentrations of lithium. Monitoring of the serum lithium level is recommended.

Digoxin

If used concomitantly, diclofenac may raise plasma concentrations of digoxin. Monitoring of the serum digoxin level is recommended.

Diuretics and antihypertensive agents

Like other NSAIDs, concomitant use of diclofenac with diuretics and antihypertensive agents (e.g., beta-blockers, angiotensin converting enzyme (ACE) inhibitors) may cause a decrease in their antihypertensive effect via inhibition of vasodilatory prostaglandin synthesis. Therefore, the combination should be administered with caution and patients, especially the elderly, should have their blood pressure periodically monitored. Patients should be adequately hydrated and consideration should be given to monitoring of renal function after initiation of concomitant therapy periodically thereafter, particularly for diuretics and ACE inhibitors due to the increased risk of nephrotoxicity.

Ciclosporin and tacrolimus

Like other NSAIDs, diclofenac may increase the nephrotoxicity of ciclosporin and tacrolimus due to the effect on renal prostaglandins. Therefore, it should be given at doses lower than those that would be used in patients not receiving ciclosporin or tacrolimus.

Drugs known to cause hyperkalaemia

Concomitant treatment with potassium-sparing diuretics, ciclosporin, tacrolimus or trimethoprim may be associated with increased serum potassium levels, which should therefore be monitored frequently.

Anticoagulants and anti-platelet agents

Caution is recommended since concomitant administration could increase the risk of bleeding. Although clinical investigations do not appear to indicate that diclofenac has an influence on the effect of anticoagulants, there are reports of an increased risk of haemorrhage in patients receiving diclofenac and anticoagulant concomitantly. Therefore, to be certain that no change in anticoagulant dosage is required, close monitoring of such patients is required.

Quinolone antibacterials

There have been isolated reports of convulsions which may have been due to concomitant use of quinolones and NSAIDs.

Other NSAIDs and corticosteroids

Co-administration of diclofenac with other systemic NSAIDs or corticosteroids may increase the risk of gastrointestinal undesirable effects (See *Warnings and Precautions*).

Anticoagulants and anti-platelet agents

Caution is recommended since concomitant administration could increase the risk of bleeding. Although clinical investigations do not appear to indicate that diclofenac affects the action of anticoagulants, there are reports of an increased risk of haemorrhage in patients receiving diclofenac and anticoagulants concomitantly. Close monitoring of such patients is therefore recommended.

Selective serotonin reuptake inhibitors (SSRIs)

Concomitant administration of SSRIs may increase the risk of gastrointestinal bleeding.

Antidiabetics

Clinical studies have shown that diclofenac can be given together with oral antidiabetic agents without influencing their clinical effect. However, there have been isolated reports of hypoglycaemic and hyperglycaemic effects necessitating changes in the dosage of the antidiabetic agents during treatment with diclofenac. For this reason, monitoring of the blood glucose level is recommended as a precautionary measure during concomitant therapy.

There have also been isolated reports of metabolic acidosis when diclofenac was coadministered with metformin, especially in patients with pre-existing renal impairment.

Methotrexate

Diclofenac can inhibit the tubular renal clearance of methotrexate hereby increasing methotrexate levels. Caution is recommended when NSAIDs (including diclofenac) are administered less than 24 hours before or after treatment with methotrexate, since blood concentrations of methotrexate may rise and result in toxicity.

Phenytoin

When using phenytoin concomitantly with diclofenac, monitoring of phenytoin plasma concentrations is recommended due to an expected increase in exposure to phenytoin.

CYP2C9 inducers

Caution is recommended when co-prescribing diclofenac with CYP2C9 inducers (such as rifampicin), which could result in a significant decrease in plasma concentration and exposure to diclofenac.

PREGNANCY AND LACTATION

Pregnancy

There are insufficient data on the use of diclofenac in pregnant women. Some epidemiological studies suggest an increased risk of miscarriage after use of a prostaglandin synthesis inhibitor (such as NSAIDs) in early pregnancy, however the overall data are inconclusive. Therefore, diclofenac should not be used during the first two trimesters of pregnancy unless the expected benefits to the mother outweigh the risks to the foetus. As with other NSAIDs, use of diclofenac during the third trimester of pregnancy is contraindicated owing to the possibility of uterine inertia, foetal renal impairment with subsequent oligohydramnios and/or premature closure of the ductus arteriosus.

Lactation

Like other NSAIDs, diclofenac passes into the breast milk in small amounts. Therefore, diclofenac should not be administered during breast feeding in order to avoid undesirable effect in the infant.

EFFECTS ON ABILITY TO DRIVE AND USE MACHINE

Patients who experience visual disturbances, dizziness, vertigo, somnolence, central nervous system disturbances, drowsiness or fatigue while taking NSAIDs should refrain from driving or operating machinery.

UNDESIRABLE EFFECTS

Summary of Safety Profile

The adverse reactions tabulated below are classified according to frequency and system organ, with either short-term or long-term use.

System Organ	Frequency	Adverse Reactions
Infection and Infestations	Very rare	Injection site abscess
Blood and lymphatic system disorders	Very rare	Thrombocytopenia, leucopenia, agranulocytosis, anaemia (including haemolytic and aplastic anaemia)
Immune system disorders	Rare Very rare	Hypersensitivity, anaphylactic and anaphylactoid reactions (including hypotension and shock) Angioneurotic oedema (including face oedema)
Psychiatric disorders	Very rare	Disorientation, depression, insomnia, nightmare, irritability, psychotic disorder
Nervous system disorders	Common Rare Very rare	Headache, dizziness Somnolence Paraesthesia, memory impairment, convulsion, anxiety, tremor, aseptic meningitis, dysgeusia, cerebrovascular accident.
Eye disorders	Very rare	Visual disturbance, blurred vision, diplopia
Ear and labyrinth disorders	Common Very rare	Vertigo Tinnitus, impaired hearing

Cardiac disorders (*data from long-term treatment with a high dose, 150 mg daily)	*Uncommon Frequency not known	Myocardial infarction, cardiac failure, palpitations, chest pain Kounis syndrome
Vascular disorders	Very rare	Hypertension, vasculitis
Respiratory, thoracic and mediastinal disorders	Rare Very rare	Asthma (including dyspnoea) Pneumonitis
Gastro-intestinal disorders	Common Rare Very rare	Nausea, vomiting, diarrhoea, dyspepsia, abdominal pain, flatulence, decreased appetite Gastritis, gastrointestinal haemorrhage, melana, haematemesis, diarrhoea haemorrhagic, gastrointestinal ulcer (with or without bleeding, gastrointestinal stenosis, or perforation, which may lead to peritonitis) Colitis (including haemorrhagic colitis, ischemic colitis and exacerbation of ulcerative colitis or Crohn's disease), constipation, glossitis, oesophageal disorder, stomatitis, intestinal diaphragm disease, pancreatitis
Hepatobiliary disorders	Common Rare Very rare	Transaminases increased Hepatitis, jaundice, liver disorder Fulminant hepatitis, hepatic necrosis, hepatic failure
Skin and subcutaneous tissue disorders	Common Rare Very rare (occasional)	Rash Urticaria Bullous eruptions, eczema, erythema, erythema multiforme, Stevens-Johnson syndrome, toxic epidermal necrolysis (Lyell's syndrome), exfoliative dermatitis, alopecia, photosensitivity reaction, purpura, Henoch-Schoenlein purpura, pruritus
Renal and urinary disorders	Very rare	Acute renal failure, haematuria, proteinuria, nephrotic syndrome, tubulointerstitial nephritis, renal papillary necrosis
General disorders and administration site conditions	Common Rare	Injection site reaction/pain/induration Oedema, injection site necrosis

Description of selected adverse drug reactions

Arteriothrombotic events

Meta-analysis and pharmacoepidemiological data point towards an increased risk of arteriothrombotic events (for example myocardial infarction) associated with the use of diclofenac, particularly at a high dose (150 mg daily) and during long-term treatment (See *Warnings and Precautions*).

OVERDOSE AND TREATMENT

Symptoms

There is no typical clinical picture resulting from diclofenac overdose. Overdosage can cause symptoms such as vomiting, epigastric pain, gastrointestinal bleeding, diarrhoea, dizziness, tinnitus or convulsions. In the event of significant poisoning, acute renal failure and liver damage are possible.

Therapeutic measures

Management of acute poisoning with NSAIDs, including diclofenac, essentially consists of supportive measures and symptomatic treatment. Supportive measures and symptomatic treatment should be given for complications such as hypotension, renal failure, convulsions, gastrointestinal disorder, and respiratory depression.

Special measures such as forced diuresis, dialysis or haemoperfusion are probably of no help in eliminating NSAIDs, including diclofenac, due to the high protein binding and extensive metabolism.

INCOMPATIBILITIES

The ampoules used IM or IV as an infusion should not be mixed with other injection solutions. Infusion solutions of sodium chloride 0.9% or glucose 5% without sodium bicarbonate as an additive present a risk of supersaturation, possibly leading to formation of crystals or precipitates. Infusion solutions other than those recommended should not be used.

STORAGE CONDITIONS

Store below 30°C and protect from light.

SHELF LIFE

Refer to packaging for shelf life.

DOSAGE FORMS AND PACKAGING AVAILABLE

Available in 100 x 3 mL glass ampoules

INSTRUCTIONS FOR USE AND HANDLING

The following directions for intramuscular injection must be followed in order to avoid damage to a nerve or other tissue at the injection site.

To be injected either intramuscularly by deep intragluteal injection into the upper outer quadrant using aseptic technique, or intravenously by slow infusion after dilution in accordance with the following instructions. Each ampoule is for single use only. The solution should be used immediately after opening. Any unused contents should be discarded.

Appropriate injection technique and length of the needle (considering the thickness of the patient's gluteal fat) should be used to avoid inadvertent subcutaneous administration of diclofenac injection.

Depending on the intended duration of infusion (see *Dosage and Administration*), mix 100 to 500 mL of isotonic saline (sodium chloride 0.9% solution) or glucose 5% solution with the contents of one diclofenac ampoule. Both solutions should be buffered with sodium bicarbonate injectable solution (0.5 mL of 8.4% or 1 mL of 4.2% or a corresponding volume of a different concentration) taken from a freshly opened container. Only clear solutions should be used. If crystals or precipitates are observed, the infusion solution should not be used.

DATE OF REVISION OF PI

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Manufacturer:

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